BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Use the individual prescription and appointment calendar. No universal tumour-marker panel or scan interval is assumed.

Check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Blood and organ safety	Full blood count, kidney and liver tests, coagulation, urine protein, thyroid-stimulating hormone, electrocardiogram (ECG) and blood pressure. Urine protein must be below 1 g/24 hours at initiation; blood pressure must be controlled.	Every 28 days: blood count, kidney tests, coagulation, urine protein and thyroid-stimulating hormone. Liver tests every 2 weeks for the first 2 months, then monthly; calcium monthly. Blood pressure after week 1, every 2 weeks in the first 2 months, then monthly. ECG when clinically indicated.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.
Appointments and medicines	Agree the treatment aim, dose and complete medicines list.	Take at about the same time each day, with or without food. Swallow capsules whole with water; do not crush or break them. A pharmacist can teach suspension preparation: place up to 5 whole capsules in a small container or 20 mL oral syringe, add 3 mL water, apple juice or milk, wait 10 minutes, then stir or shake for 3 minutes. Take all the mixture, add another 2 mL liquid and take the rinse. For a feeding tube use water only. If a missed dose cannot be taken within 12 hours, skip it and take the next dose at the usual time.	Keep a current dose and symptom record.	Treat diarrhoea and vomiting promptly using the agreed plan because dehydration can damage the kidneys. Check blood pressure as taught. Report sudden severe headache, vision changes, bleeding, chest pain, severe abdominal pain or fluid leakage from an abnormal opening urgently.	Keep follow-up and recovery checks even after the last dose.
Cancer-specific and specialist tests	Thyroid-stimulating hormone checks the hormone-suppression plan. Urine protein and blood calcium are safety checks. Thyroglobulin and response imaging follow the individual thyroid cancer plan.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.	Agree the named follow-up team and remaining checks.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00295 v6: official regimen

Tests in your plan

Baseline bloods and tests

WHEN

Full blood count, kidney and liver tests, coagulation, urine protein, thyroid-stimulating hormone, ECG and blood pressure. Urine protein must be below 1 g/24 hours at initiation; blood pressure must be controlled.

Checks organ safety and suitability before treatment.

Treatment safety monitoring

WHEN

Every 28 days: blood count, kidney tests, coagulation, urine protein and thyroid-stimulating hormone. Liver tests every 2 weeks for the first 2 months, then monthly; calcium monthly. Blood pressure after week 1, every 2 weeks in the first 2 months, then monthly. ECG when clinically indicated.

Finds changes that may need supportive care, a dose hold or adjustment.

Specialist and cancer tests

If part of your plan

WHEN

Thyroid-stimulating hormone checks the hormone-suppression plan. Urine protein and blood calcium are safety checks. Thyroglobulin and response imaging follow the individual thyroid cancer plan.

Distinguishes treatment selection, hormone monitoring and cancer response.

THE WHOLE TREATMENT JOURNEY

Lenvatinib: differentiated thyroid cancer: treatment and checks

Progressive, locally advanced or metastatic differentiated thyroid cancer that is refractory to radioactive iodine.

Lenvatinib 24 mg once daily, continuously, until progression or unacceptable side effects. The clinician may prescribe a lower starting dose or later reductions.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Blood and organ safety NCCP	Full blood count, kidney and liver tests, coagulation, urine protein, thyroid-stimulating hormone, ECG and blood pressure. Urine protein must be below 1 g/24 hours at initiation; blood pressure must be controlled.	Every 28 days: blood count, kidney tests, coagulation, urine protein and thyroid-stimulating hormone. Liver tests every 2 weeks for the first 2 months, then monthly; calcium monthly. Blood pressure after week 1, every 2 weeks in the first 2 months, then monthly. ECG when clinically indicated.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.
Appointments and medicines NCCP	Agree the treatment aim, dose and complete medicines list.	Take at about the same time each day, with or without food. Swallow capsules whole with water; do not crush or break them. A pharmacist can teach suspension preparation: place up to 5 whole capsules in a small container or 20 mL oral syringe, add 3 mL water, apple juice or milk, wait 10 minutes, then stir or shake for 3 minutes. Take all the mixture, add another 2 mL liquid and take the rinse. For a feeding tube use water only. If a missed dose cannot be taken within 12 hours, skip it and take the next dose at the usual time.	Keep a current dose and symptom record.	Treat diarrhoea and vomiting promptly using the agreed plan because dehydration can damage the kidneys. Check blood pressure as taught. Report sudden severe headache, vision changes, bleeding, chest pain, severe abdominal pain or fluid leakage from an abnormal opening urgently.	Keep follow-up and recovery checks even after the last dose.

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Cancer-specific and specialist tests NCCP	Thyroid-stimulating hormone checks the hormone-suppression plan. Urine protein and blood calcium are safety checks. Thyroglobulin and response imaging follow the individual thyroid cancer plan.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.	Agree the named follow-up team and remaining checks.

What each check is for

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Treatment and review

Once daily, continuously

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Ask / act: Every 28 days: blood count, kidney tests, coagulation, urine protein and thyroid-stimulating hormone. Liver tests every 2 weeks for the first 2 months, then monthly; calcium monthly. Blood pressure after week 1, every 2 weeks in the first 2 months, then monthly. ECG when clinically indicated.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-14 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.50